



HISTORICAL DEVELOPMENT OF ETHIOPIA'S HEALTH CARE SYSTEM, HEALTH SECTOR DEVELOPMENT PROGRAM I

Team-2

Moderator: Adiam Nega

INTRODUCTION

Ethiopia's healthcare system has undergone a remarkable evolution over the past century





From relying on traditional medicine to implementing structured, government-led initiatives like the Health Sector Development Program I (HSDP I), the country has made important strides in expanding access to health services

HSDP I was the first major strategic policy document to address systemic weaknesses in health infrastructure, human resources, and service delivery, with the goal of operationalizing the 1993 National Health Policy.



Historical Development: Pre-Modern Period

Reliance on traditional healing practices:

- Herbal preparations, bone setting, religious rituals.
- Indigenous health practices.

Traditional healers: Highly respected, managed broad health problems.

Limitations: Lack of scientific understanding, institutional organization, scalability.





Historical Development: Imperial Period (Early 1900s–1974)

Introduction of modern medicine:

- First hospital established (Emperor Menelik II, 1908).
- Ministry of Public Health created (1948).

Challenges: Urban concentration of services, rural inaccessibility.

Early efforts: Facility building, personnel training, sanitation.

Outcome: Fragmented, elite-focused services.





Historical Development: Derg Regime (1974–1991)

Post-1974 revolution: Nationalization of health services, socialist principles.

Introduction of free healthcare.

Expansion of rural access: Health stations, mobile units, community outreach.

Adoption of Primary Health Care (PHC) model (Alma-Ata Declaration).

Challenges: Civil unrest, prolonged wars, economic decline weakened the sector.



Historical Development: Post-1991 Era & 1993 National Health Policy

New federal government prioritized public health reform.

1993 National Health Policy:

- Focus on preventive and promotive care.
- Decentralization, equity, community participation.

Paved the way for Health Sector Development Programs (HSDPs).

HSDP I: Initiated in 1998 as the first five-year plan to operationalize the policy.



HSDP I (1998–2002): Background and Rationale

Context (1990s): Systemic challenges, profound health inequities.

Grim National Health Indicators:

- Health service access: 42%.
- Maternal mortality: 871 per 100,000 live births.
- Under-five mortality: 204 per 1,000 live births.
- Outpatient utilization: 0.27 visits per capita annually.

Rationale: Urgent need for fundamental transformation, comprehensive national health strategy.

Holistic Approach: Decentralization, infrastructure, workforce training, community-based interventions.



HSDP I: Strategic Objectives (Part 1)

1. Expand Access to Essential Primary Healthcare (PHC) Services:

- Strategic infrastructure development (health posts, centers).
- Targeted staffing in rural/remote areas.
- Equipping facilities, establishing referral systems.

2. Reduce Morbidity and Mortality:

- Focus on communicable diseases, maternal and child health.
- Comprehensive prevention and treatment strategies.
- Vaccination programs, antenatal/postnatal care, disease surveillance.



3. Improve Drug Availability and Rational Use:

Centralized procurement and distribution (PFSA).

Consistent, affordable, high-quality pharmaceutical access.

Robust supply chain, quality control, national drug policy.

4. Increase the Number and Quality of Health Personnel:

Comprehensive training programs, scholarships.

Equitable deployment strategies.

Continuous professional development, retention incentives.



HSDP I: Strategic Objectives (Part 3)

5. Strengthen Health System Governance:

- Robust planning, transparent budgeting, rigorous monitoring.
- Data-driven management, accountability mechanisms.
- Culture of continuous improvement, evidence-based decision-making.

6. Engage Communities:

- Enhance accountability, ownership, sustainability.
- Community health committees, participatory health education.
- Empowering local leaders, feedback mechanisms.



HSDP I: Key Components - Health Infrastructure Development

Goal: Strategically expand health service coverage, address gaps in rural/underserved regions.

Achievements (by 2002):

- 107 health centers constructed.
- 2,899 health posts constructed.

Focus: Geographically prioritized approach, targeting regions with lowest health indicators.

Impact: Transformative expansion, improved access in remote communities.



HSDP I: Key Components - Human Resource Development

Comprehensive Strategies:

- Expanded and diversified training programs for health professionals.
- Focus on Health Officers, Midwives, Nurses, Sanitarians.

New Institutions: Established in emerging regions to recruit local talent.

In-service Training: Mentorship, refresher courses, technology-enhanced CPD.

Aim: Increase quantity and quality of healthcare workers, retain talent.



HSDP I: Key Components - Essential Medicines and Equipment

Pharmaceutical Management Strategy (PFSA):

- Centralized procurement for efficiency and cost reduction.
- Ensured consistent medicine availability.

Improved Storage & Distribution:

- Modern regional depots with temperature control, tracking tech.
- Minimized transportation time, reduced degradation.

Standardized National Treatment Guidelines:

- Promoted rational drug use, evidence-based protocols.
- Reduced inappropriate use, controlled antimicrobial resistance.



HSDP I: Key Components - HMIS & Quality Assurance

Health Management Information Systems (HMIS):

- Standardized data collection tools and reporting templates.
- Baseline assessments, training of HMIS focal persons.
- Development of early warning systems for disease surveillance.

Quality Assurance and Supervision:

- Supportive supervision practices (on-site visits, mentorship).
- Peer-to-peer learning platforms.
- Protocols for facility audits, checklists, performance monitoring.
- Identified gaps, provided targeted support.



HSDP I: Implementation Strategies (Part 1)

Decentralization:

- Shifted decision-making and resource management to regional/district levels.
- Empowered Regional Health Bureaus, tailored interventions.
- Increased responsiveness to local priorities.
- Fostered accountability and transparency.

Sector-Wide Approach (SWAp):

- Harmonized donor and government funding.
- Pooled resources, better coordination, efficient utilization.
- Common priorities, strategies, performance indicators.
- Greater flexibility in resource allocation.



HSDP I: Implementation Strategies (Part 2)

Community Engagement:

- Health committees at kebele (village) level.
- Platform for community voice, participation in decision-making.
- Training of community health volunteers and traditional birth attendants (TBAs).
- Delivery of basic services, health education.

Public-Private Partnerships (PPPs):

- Engaged faith-based organizations and private sector facilities.
- Expanded reach and delivery of essential services.
- Leveraged existing infrastructure, expertise, community connections.
- Broadened geographic coverage and service range.



HSDP I: Implementation Strategies (Part 3)

Monitoring and Evaluation (M&E):

- Rigorous M&E for effectiveness and continuous improvement.
- Quarterly and annual review processes.
- Comprehensive set of performance indicators.
- Agile approach to respond to emerging issues.
- Holistic assessment of impact, informed policy decisions.



HSDP I: Achievements

Increased Health Access: Service coverage rose from 42% to 64% by 2002.

Immunization & Disease Control:

- DPT3 vaccination coverage: 36% to 51%.
- 1.5+ million insecticide-treated nets (ITNs) distributed (malaria).
- Expansion of TB detection and treatment centers.

Maternal & Child Health:

- Antenatal care (ANC) coverage: 27% to 42%.
- Modest improvement in skilled birth attendance.
- Growth in family planning uptake.

Human Resource Gains: Workforce grew from 13,000 to over 21,000 professionals.

System Strengthening: Standardized treatment protocols, nationwide HMIS rollout.



HSDP I: Challenges

Persistent Inequities: Urban vs. rural areas, lack of staffing/facilities in remote communities.

Financial Constraints: Low per capita health budget (~USD 4.5), heavy reliance on donor funding.

Human Resource Issues: High staff turnover, limited incentives, urban concentration.

Weak Supply Chain: Stock-outs of essential drugs (poor forecasting, transport limitations).

Weak Health Information System: Incomplete and untimely data reporting.



Conclusion:

HSDP I was a pivotal transformation, tackling systemic deficiencies and laying a crucial foundation for subsequent reforms. Ethiopia's health system continues to evolve, driven by commitment to equity, quality, and sustainability.